



When to Provide Virtual Care

Clinical Case Quiz



A 25-year-old MSM (men who have sex with men) patient presents via phone visit with concerns about contracting a sexually transmitted infection. He reports mild dysuria and believes he may have a slight penile discharge. He informs you that he had unprotected insertive anal intercourse (as a top) with a male partner approximately 5 days ago. His sexual partners are only men, and he reports engaging in both insertive and receptive intercourse at times (he considers himself “vers”). He reports that the sexual partner was anonymous—“We met through the iPhone application

Grindr. He told me *he* was last tested for STIs about 1 month ago and was negative at that time.” Your patient is asking to be screened for any potential infections he may have contracted.

On review of this member’s medical record, you notice that he is healthy and currently taking PrEP for HIV prevention. He is current with this medication and, when asked, he tells you that he has not missed any doses. His last screening for STIs was one month ago. He has a history of a chlamydia infection (6 months ago) and was treated for syphilis 3 years ago.

What is the most appropriate next step?

- a. Order a complete STI screening panel, including HIV, syphilis, Hepatitis C, as well as gonorrhea and chlamydia via 3-point testing that includes urine, oral, and rectal swabs.
- b. Counsel this patient about the latency period for STIs and the importance of repeat testing for high-risk scenarios.
- c. Discuss DoxyPEP with this member, informing him that he may be a great candidate for this medication, and that it may be helpful as a strategy to prevent gonorrhea, chlamydia, and syphilis infections for future sexual encounters.
- d. Counsel this member about safe sexual practices and that, while PrEP and DoxyPEP are effective strategies to prevent some STIs, others, such as genital warts and genital herpes, may still be contracted without adequate protection, such as condom usage.
- e. All of the above.

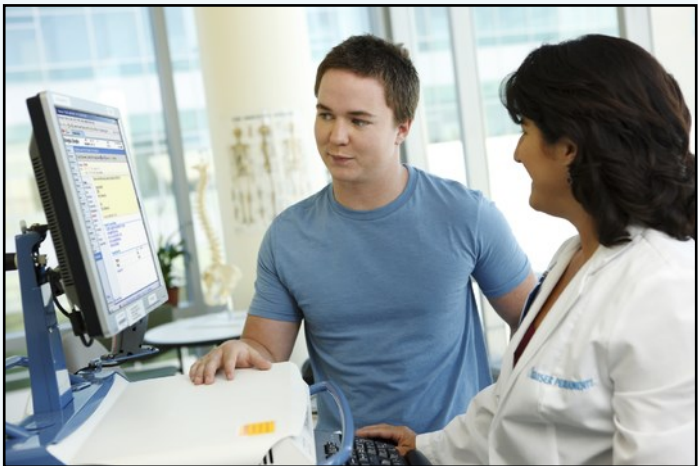
The answer is e.

Discussion

Many sexually transmitted infections, including syphilis, gonorrhea, and chlamydia, are on the rise in the United States. From 2018 to 2022, reported cases of syphilis rose 80% in the U.S., with the highest numbers reported since the 1950s. There are many factors that may be contributing to this rise. Social factors play a key role, as there is a certain stigma and discomfort around discussions that involve sex with a patient’s medical provider. Many medical providers take insufficient sexual histories with their patients, which leads to missed opportunities for counseling and testing for STIs.

Access to care is also a concern. There aren’t enough available clinics that provide free STI testing, and many existing clinics are seeing a lack of funding needed to adequately screen their patients. Other factors, such as lack of consistent condom use, cultural beliefs, having multiple sexual partners, drugs, alcohol abuse, and an underemphasis on sexual health practices all are contributory.

Engaging with a patient who is asking for STI testing may be used as an opportunity to implement the CDC’s current strategy of “Talk. Test. Treat.” The first step is to take a thorough sexual history to identify the patient’s risk. Be sure that this history includes things such as sexual orientation, sexual practices, any history of previous STIs, and strategies the patient uses to prevent STIs. “The 5 Ps” of sexual history taking may be a helpful reminder to Providers—**Partners. Practices. Protection from STIs. Past history of STIs. Pregnancy intention.**



When a member is asking to be tested, be sure to ask if it is a routine test or if they feel they have been recently exposed to an STI. If they may have been exposed to HIV in the previous 72 hours, a discussion around PEP, or Post-Exposure Prophylaxis for HIV, should occur. When testing, be sure that the test includes blood, urine, and swab testing (when appropriate) so that all commonly seen STIs can be appropriately screened for.

It is important that members know that there is a latency period between being exposed to an STI and when the test for an STI becomes positive. This can be 2-10 days for STIs such as gonorrhea and chlamydia, 2-4 weeks for HIV (and, in some cases, up to 2 months), and sometimes 1-2 months for conditions such as syphilis and hepatitis. Therefore, it is important to counsel the patient about the importance of re-testing for high-risk scenarios and exposures.

In addition to PrEP for HIV prevention, we now have DoxyPEP as an additional tool that may prevent gonorrhea, chlamydia, and syphilis. The CDC recommends that MSM and TGW (transgender women, i.e., someone who was assigned male at birth but identifies now as female) who have had a bacterial STI diagnosed in the last 12 months (specifically syphilis, chlamydia, and gonorrhea) should receive counseling that DoxyPEP can be used as post-exposure prophylaxis to prevent these infections. Clinical judgment should be used to recommend DoxyPEP to other MSM and TGW patients, depending on their risk factors.

Any primary care provider can prescribe DoxyPEP. The prescription is written as Doxycycline Monohydrate 100 mg tablets, with the direction to take 2 tablets at the same time within 72 hours after sexual intercourse for STI prevention. 30 pills may be dispensed at a time with 3 refills. Further refills can be managed by their primary care physician. Unlike PrEP for HIV prevention, in which routine labs are needed every 3 months, there are no such requirements for DoxyPEP.

It is also important to inform members that, while DoxyPEP (as well as PrEP) are great strategies that mitigate risk of acquiring STIs, they do not protect against **all infections**. Therefore, we recommend having open-ended discussions free of judgment and personal bias with our members to encourage them to get tested for STIs routinely based on their own sexual practices, and to have open discussions about STI testing with their sexual partners.

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